

Patient ID: EGPatient ID: EGPeriod: March 15 to May 6, 2024Report Date: May 7, 2024Coverage: Bed: 915/1245h (73.5%) | Chair: 670/1245h (53.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient's Clinical Status Over the Period

Mixed Instability (Moderate HR) Mar 17 to Mar 18 (2d)	Mixed Instability (Low HR) Mar 27 to Mar 29 (3d)	Mixed Instability (Moderate HR) Apr 02 to Apr 04 (3d)	Mixed Instability (Low HR) Apr 12 to Apr 13 (2d)	Mixed Instability (Moderate HR) Apr 22 to Apr 26 (5d)
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SECTION 1 — Clinical Intelligence: Signal Summary

Over 53 days of monitoring, 1585 of 1245 expected hours were recorded (103.5% coverage). 51 episodic events spanning 60 total hours were detected, including: Elevated Breathing Rate, Low Heart Rate, Very Low Heart Rate. The clinical trajectory showed 5 distinct phases, progressing from mixed instability (moderate hr) in Mar 17 (avg 62 bpm) to mixed instability (moderate hr) by Apr 26 (avg 60 bpm). The period showed 5 distinct phases. Mixed Instability (Low HR) (Mar 27 to Mar 29): Mixed episode pattern predominantly featuring lower heart rates (average 58 bpm, falling below the patient's baseline). Mixed Instability (Moderate HR) (Apr 02 to Apr 04): Mixed episode pattern with fluctuating heart rates (average 61 bpm). Mixed Instability (Low HR) (Apr 12 to Apr 13): Mixed episode pattern predominantly featuring lower heart rates (average 59 bpm, falling below the patient's baseline). Mixed Instability (Moderate HR) (Apr 22 to Apr 26): Mixed episode pattern with fluctuating heart rates (average 60 bpm). 1 additional phases of lower clinical concern were detected during the monitoring period.

Trend Assessment: Intermittently unstable vital sign pattern

Overall Action Posture: Closer clinical observation warranted

Suggested Clinical Review Actions

- Mixed Instability (Moderate HR) (Apr 02 to Apr 04): Mixed episodes including Low Heart Rate. Correlate with clinical context, medication timing, and symptom assessment.
- Mixed Instability (Moderate HR) (Apr 22 to Apr 26): Mixed episodes including Low Heart Rate. Correlate with clinical context, medication timing, and symptom assessment.
- Overall trajectory shows 5 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

SECTION 2 — High Priority Episodic Events

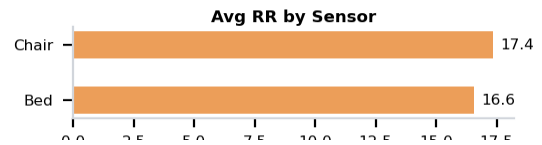
Event Type	Date / Time Window	Dur	N/D	HR	RR	Comments
Very Low Heart Rate	03/29 4:00 AM to 03/29 5:00 AM	2h	N	avg 43 / min 41	avg 12 / min 12	Sustained deviation; review context
Very Low Heart Rate	03/29 7:00 AM (1h)	1h	D	avg 44 / min 44	avg 11 / min 11	Sustained deviation; review context
Low Heart Rate	03/29 2:00 AM to 03/29 3:00 AM	2h	N	avg 47 / min 44	avg 12 / min 12	Brief deviation; continue monitoring
Low Heart Rate	03/18 3:00 PM (1h)	1h	D	avg 49 / min 47	avg 15 / min 15	Brief deviation; continue monitoring
Low Heart Rate	03/24 3:00 PM (1h)	1h	D	avg 50 / min 46	avg 17 / min 17	Brief deviation; continue monitoring
Low Heart Rate	03/26 8:00 AM (1h)	1h	D	avg 49 / min 48	avg 18 / min 18	Brief deviation; continue monitoring

SECTION 3 — Heart Rate and Breathing Rate Statistics

Metric	Mean	Min	Max	P5	P95
Heart Rate Avg (bpm)	60.4	42.5	87.6	53.0	69.8
Heart Rate Min (bpm)	55.4	41.1	86.6	48.7	64.6
Heart Rate Max (bpm)	66.4	43.4	91.7	55.9	76.5
Avg RR (breaths/min)	16.9	2.1	46.0	11.9	21.7
Min RR (breaths/min)	11.4	2.1	46.0	6.2	17.0
Max RR (breaths/min)	23.3	2.1	49.8	14.8	33.9

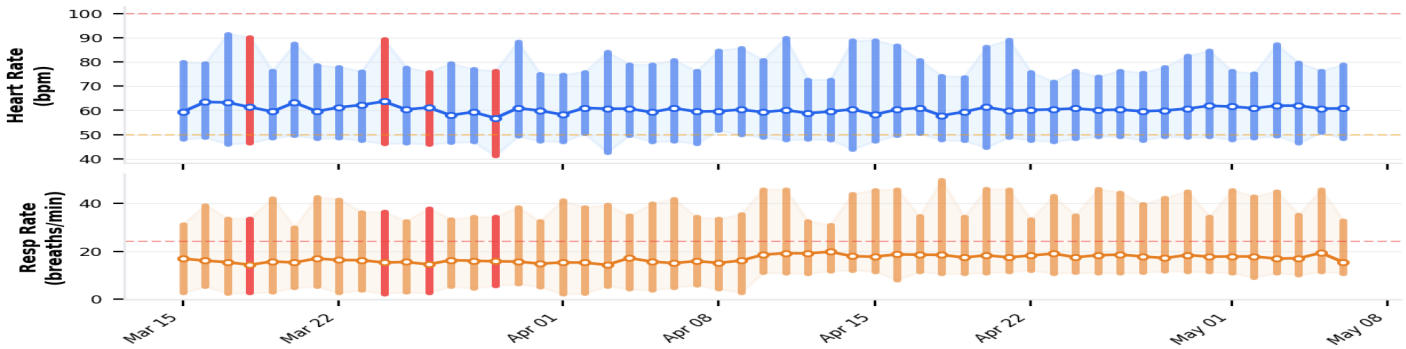
SECTION 4 — Positional Sensor Breakdown

Sensor Location	Avg HR (bpm)	Avg RR	Hours Monitored
Bed	62.1	16.6	915
Chair	58.1	17.4	670

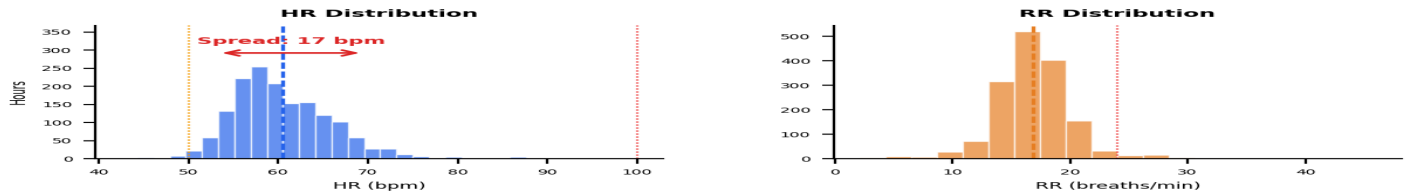


SECTION 5 — Visual Trend Analysis

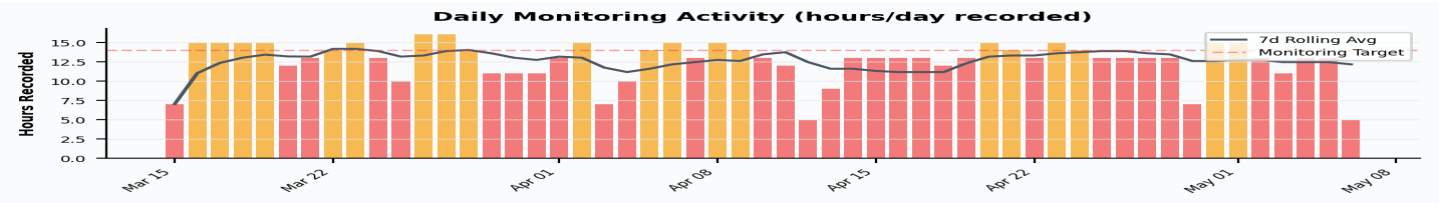
Daily Vital Sign Ranges (Candlestick Trend)



Vital Sign Distribution (P5 to P95 Variability)



SECTION 6 — Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.